

Chemotherapy for ovarian cancer

You may have chemotherapy on its own, or with surgery, to treat ovarian cancer, fallopian tube cancer or primary peritoneal cancer.

On this page

- [What is chemotherapy?](#)
- [When chemotherapy is used for cancer of the ovary, fallopian tube or peritoneum](#)
- [Chemotherapy drugs used for cancer of the ovary, fallopian tube or peritoneum](#)
- [Treating ovarian cancer that comes back](#)
- [Having chemotherapy](#)
- [Side effects of chemotherapy](#)
- [How we can help](#)

What is chemotherapy?

Chemotherapy uses anti-cancer (cytotoxic) drugs to destroy cancer cells. It is one of the main treatments for [cancer of the ovary](#), [fallopian tube cancer](#) or [primary peritoneal cancer](#).

When chemotherapy is used for cancer of the ovary, fallopian tube or peritoneum

You usually have chemotherapy with [surgery](#) or sometimes on its own.

It can be given :

- after surgery – to reduce the risk of the cancer coming back (adjuvant chemotherapy)
- before surgery – to shrink the cancer to make the operation more effective (neo-adjuvant chemotherapy)
- to treat any cancer that cannot be removed during surgery
- as your main treatment – if your doctors think surgery is not the best treatment option for you, or if you are not well enough to have a big operation.

Chemotherapy after surgery

Chemotherapy starts when you have recovered from surgery. This is usually about 4 weeks after surgery. But it can be up to 12 weeks after.

If you have [stage 1 ovarian cancer](#), you may not have chemotherapy. It can depend on the type, sub-stage and grade of the cancer. Your doctor can talk to you about this.

Chemotherapy before surgery

If you are having chemotherapy before surgery, you usually have it every 3 weeks. This is called a treatment cycle. You will have 3 treatment cycles.

After 3 treatment cycles, you will have a [CT scan](#) to check how well the chemotherapy has worked. Your doctors will tell you if the cancer has shrunk enough for you to have surgery. If you do not have surgery, you will have another 3 cycles of chemotherapy (6 cycles in total).

Chemotherapy during surgery (HIPEC)

You may have a chemotherapy drug called cisplatin during surgery. It is given through plastic tubes that go into the abdomen (tummy area). This is called hyperthermic intraperitoneal chemotherapy or HIPEC. The chemotherapy is heated and left inside the tummy for 90 minutes. It is drained before the surgery has finished, and the tubes are removed.

HIPEC can help control the cancer. The side effects can be different from chemotherapy given into a vein. Your cancer doctor can tell you more about this. You will usually have 3 more cycles of chemotherapy after you have recovered from surgery.

Chemotherapy drugs used for cancer of the ovary, fallopian tube or peritoneum

You usually have a combination of the chemotherapy drugs [carboplatin and paclitaxel](#).

You may have carboplatin on its own. This may be because you are not well enough to cope with the side effects of a paclitaxel and carboplatin combination. Carboplatin is sometimes given on its own for early stage ovarian cancer.

You may have chemotherapy with a targeted therapy drug called [bevacizumab](#).

Other chemotherapy drugs used to treat ovarian cancer include:

- [cisplatin](#)
- [docetaxel](#)
- [paclitaxel](#)
- [etoposide](#)
- [gemcitabine](#)
- [pegylated liposomal doxorubicin](#) (sometimes called Caelyx[®]).

Treating ovarian cancer that comes back

If ovarian cancer comes back, chemotherapy can usually control it for a period of time. If your first treatment with carboplatin or cisplatin worked, you may have the same combination of drugs again. Or you may have other drugs such as pegylated liposomal doxorubicin or gemcitabine.

If the cancer comes back soon after chemotherapy treatment, you will have different chemotherapy drugs.

Having chemotherapy

You usually have chemotherapy as an outpatient in a hospital chemotherapy day unit. You usually have the drugs into a vein (intravenously).

How often you have chemotherapy

You have chemotherapy as 1 or more sessions of treatment. Each session takes a few hours. After the session, you will have a rest period of a few weeks. Together, a chemotherapy session and rest period is called a cycle.

The length of a cycle depends on the chemotherapy drugs you have. But most cycles are 1 to 4 weeks long. Your cancer doctor, specialist nurse or chemotherapy nurse will explain what to expect. Most courses of chemotherapy are 6 cycles.

If you are having bevacizumab with chemotherapy, you may continue having bevacizumab after chemotherapy has ended.

Side effects of chemotherapy

Chemotherapy drugs may cause side effects. But these can usually be controlled with medicines and will usually go away after treatment has finished.

Not all drugs cause the same side effects for each person. Some people have very few side effects. Your cancer doctor or specialist nurse will explain what to expect.

Some general side effects of chemotherapy include:

- a higher risk of getting an [infection](#)
- more risk of bleeding or bruising
- [feeling tired](#)
- [feeling sick](#) (nausea)
- [diarrhoea](#)
- hair thinning or [hair loss](#)

We have more information about the [side effects of chemotherapy](#).

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Below is a sample of the sources used in our ovarian cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

Ovarian cancer: recognition and initial management. Clinical guideline [CG122] Published: 27 April 2011
Last updated: 02 October 2023 www.nice.org.uk/guidance/cg122 (accessed May 2024)

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

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Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help